ICARB-D ANNUAL MATURITY SURVEY

Dental School Departments and Courses

Artificial intelligence · Health informatics · Robotics · Bioinformatics

Canada · United States · Brazil · China · India

Department/course-specific annual questionnaire · version 2.0.0 · 2026-10-28

Proposed article title

Mapping the Digital Maturity of Dental Education: An Annual, Course-Level Survey of Artificial Intelligence, Health Informatics, Robotics and Bioinformatics in Canada, the United States, Brazil, China and India

Built on the PAHO/WHO–IDB AI Readiness Assessment Toolkit (2024), AI-GUARD (2026), Bias-free AI in Health (2026) and AI Prompt Design for Public Health (2025).

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1. About the study

The ICARB-D annual survey gives a comprehensive picture of how mature dental school departments and courses are in four technology domains: artificial intelligence (AI), health informatics, robotics and digital dentistry, and bioinformatics. It is run by the International Consortium for AI, Robotics, and Bioinformatics in Dentistry (ICARB-D), whose main goal is to create a central point of collaboration and to build a free, open AI model for dentistry through joint work.

The survey pictures the current physical resources of each department or course (computers, AI tools, robotic and digital equipment, laboratories, datasets) and its human resources (faculty able to teach, researchers, technical staff, partners). It also measures teaching hours by topic, including programming and formal AI development, generative-AI policy, governance, implementation and sustainability, and each unit's capacity to contribute to the open AI model.

The survey is repeated every year. Each annual wave maps the state of the art across universities and courses; linking waves for the same department shows how it develops over time. Results are consolidated from course to department, school and country.

The dimensions and 1–4 maturity scale are adapted from the PAHO/WHO–IDB AI in Public Health Readiness Assessment Toolkit. Governance items for AI tools follow the PAHO AI-GUARD tool (Yes / Partial / No), bias items follow PAHO's Bias-free AI in Health guidance, and generative-AI items follow PAHO's AI Prompt Design for Public Health.

Objectives

- Describe the physical resources of each named department/course: computing, AI tools, digital and robotic equipment, laboratories, datasets, connectivity and security.
- Describe human resources: faculty able to teach each domain, researchers, training received, technical staff and interdisciplinary partners.
- Quantify teaching: hours per year by topic for AI (including programming and model development), health informatics, robotics/digital dentistry and bioinformatics, at undergraduate, graduate and residency level.
- Assess governance, ethics, bias, generative-AI policy and regulatory alignment, using country-specific references.
- Assess implementation, evaluation and financial sustainability.
- Map each unit's interest and capacity to contribute data, annotation and validation to a free, open dental AI model.
- Build a maturity index per domain for each department, school and country, and track its change from year to year.

How the questionnaire is applied

- **Step 1.** Each year, the research team contacts the department head or course director and completes Part A (profile, survey year and consent) with them. Saving Part A on the study website generates a unique 6-character access code (format XXX-XXX). If the unit answered in a previous year, its previous code is recorded to link the waves.
- **Step 2.** The team sends the code and the website address to the respondent.
- **Step 3.** The respondent enters the code on the website (the only field on the login page) and answers Parts B–M. Answers are saved at every section change, so the form can be completed in several sessions. A public guide explains every question and what it captures.
- **Step 4.** After submission the form is locked. Each form produces one row in the data file (responses.csv), one column per item, ready for statistical analysis.

Respondent instructions

- Answer every item only for the department/course named in Part A. Do not answer for the school as a whole unless an item says so.
- Where an item asks about a course, write the course code and title.
- If the same department runs more than one relevant course, a separate form (and code) is issued for each named course.
- Use "Don't know / Not applicable" rather than guessing. Approximate numbers are acceptable.
- Hours are reported per academic term. Item B05 (terms per year) converts them into hours per year; programs with an annual system (e.g., BDS in India) enter 1 term per year.

Country context

Items marked "Country adaptation" are read with the national references below. Option lists that are specific to a country are tagged with the country code (CA, US, BR, CN, IN).

	Canada	United States	Brazil	China	India
Degree	DDS/DMD	DDS/DMD	Bacharelado em Odontologia (Cirurgião-Dentista)	Bachelor of Medicine in Stomatology (口腔医学, 5-year) or integrated 8-year program	BDS (Bachelor of Dental Surgery)
Curriculum / accreditation body	CDAC (Commission on Dental Accreditation of Canada)	CODA (Commission on Dental Accreditation)	MEC/INEP (DCN Odontologia, Res. CNE/CES 3/2021) and CAPES for graduate programs	Ministry of Education (MOE) program accreditation	Dental Council of India (DCI) / National Dental Commission (NDC Act 2023), NAAC
Professional and regulatory bodies	CDA, ACFD and provincial regulatory colleges (e.g., RCDSO, ODQ)	ADA, ADEA and state dental boards	CFO and regional councils (CROs)	Chinese Stomatological Association (CSA) and National Health Commission (NHC)	Indian Dental Association (IDA) and state dental councils
Privacy law	PIPEDA and provincial health-privacy laws (e.g., PHIPA, Quebec Law 25)	HIPAA and FERPA	LGPD (Lei 13.709/2018)	Personal Information Protection Law (PIPL, 2021) and Data Security Law (2021)	Digital Personal Data Protection Act 2023 (DPDP) and its Rules
AI device regulator	Health Canada	FDA	ANVISA	NMPA (National Medical Products Administration)	CDSCO (Medical Devices Rules 2017)
Academic calendar	Most programs use two semesters per year.	Programs use semesters, trimesters or quarters.	Most programs use two semesters per year.	Most programs use two semesters per year.	BDS commonly follows an annual (yearly) examination system; enter 1 term per year in that case.
Languages	English / French	English	Portuguese	Chinese (Mandarin)	English (plus regional languages)

Readiness domains and scale

Maturity items use a 1–4 anchored scale adapted from the PAHO/WHO toolkit (1 = absent, 2 = initial, 3 = developing, 4 = established), plus "Don't know / Not applicable" (coded −1 and treated as missing). Each 1–4 item belongs to one of ten domains:

- **INF —** Physical infrastructure and technology
- **DAT —** Data management, quality and privacy
- **HUM —** Human resources and expertise
- **CUR —** AI curriculum and teaching
- **GEN —** Generative AI governance and literacy
- **HIN —** Health informatics
- **ROB —** Robotics and digital dentistry
- **BIO —** Bioinformatics and omics
- **GOV —** Governance, ethics and regulation
- **IMP —** Implementation, evaluation and sustainability

2. Questionnaire

Part A. Department/course profile (completed by the research team)

What this part captures. Identifies exactly which unit is being described (country, school, department, course, level) and records consent. These fields are the keys used to group answers by department, school and country, and to follow the same unit from one annual wave to the next.

The research team completes this part with the department head or course director (by phone, video call or email). Saving it generates the XXX-XXX access code given to the respondent.

A01. Country. Country *[required]*

Captures: Country of the school. Used for country comparisons and to adapt examples (laws, regulators, degree names) in later items.

- ☐ Canada
- ☐ United States
- ☐ Brazil
- ☐ China
- ☐ India

A02. Region. State / province / region *[required]*

Captures: Sub-national region, to describe geographic spread and regional inequalities within a country.

Answer: _____

A03. Institution. Name of dental school (institution) *[required]*

Captures: The school. All departments of the same school are consolidated into one school score.

Answer: _____

A04. Institution type. Type of institution *[required]*

Captures: Funding model of the school, to test whether public and private schools differ in maturity and resources.

- ☐ Public (federal / national)
- ☐ Public (state / provincial / municipal)
- ☐ Private, non-profit
- ☐ Private, for-profit

A05. City. City *[required]*

Captures: City of the campus, for mapping and to distinguish campuses of the same institution.

Answer: _____

A06. Department. Name of the department/course being reported (e.g., Department of Oral Radiology) *[required]*

Captures: The department. Answers are first consolidated at this level.

Answer: _____

A07. Course. Course code and title referenced in this form (e.g., "RAD-301 Oral Radiology II"). Write "Department-wide" if the form covers the whole department. *[required]*

Captures: The named course, so readers can see exactly which courses produced each school's curriculum results.

Answer: _____

A08. Level. Level of this named department/course *[required]*

Captures: Training level, harmonized across five national systems, to compare undergraduate, graduate and residency teaching.

□ Undergraduate first professional degree (DDS/DMD, BDS, Bacharelado em Odontologia, 口腔医学 Bachelor)

□ Research master's (MSc/MS, Mestrado acadêmico, 学术型硕士)

□ Professional/clinical master's or specialty degree (MDS in India, Mestrado profissional, 专业型硕士)

□ Doctoral (PhD/DSc, Doutorado, 博士)

□ Residency / specialty training (US/Canada residency, Brazil residência/especialização, China standardized residency training 规培)

□ Integrated long-track program (e.g., China 8-year program, DDS-PhD)

□ Continuing education / CPD

□ Other

Other, specify: _____

A09. Respondent role. Role of the respondent in this named department/course *[required]*

Captures: Who answered, to judge how close the respondent is to the information (used in sensitivity analyses).

□ Department head / chair

□ Course director / coordinator

□ Program director (graduate / residency)

□ Curriculum committee member

□ Faculty representative

□ Dean / associate dean

□ Other

Other, specify: _____

A10. Respondent name. Respondent full name *[personal data — stored separately]*

Captures: Contact only. Stored in a separate private file, never in the analysis data.

Answer: _____

A11. Respondent email. Respondent email *[personal data — stored separately]*

Captures: Contact only, for reminders and next year's invitation. Stored separately.

Answer: _____

A12. Interviewer. Research team member who completed Part A *[personal data — stored separately]*

Captures: Audit trail of who issued the code. Stored separately.

Answer: _____

A13. Language. Preferred language of the respondent

Captures: Language for communication and translated versions of the instrument.

□ English

□ French

□ Portuguese

□ Chinese

□ Hindi

□ Spanish

□ Other

Other, specify: _____

A14. Consent. Informed consent to participate was obtained *[required]*

Captures: Ethical requirement. Forms without consent are excluded from analysis.

- ☐ Yes
- ☐ No

A15. Survey year (wave). Survey year (annual wave), e.g., 2026 *[required]*

Captures: The annual wave. The survey is repeated every year so changes in each department can be tracked over time.

Answer (undefined): _____

A16. Previous code. Access code this same department/course used in the previous year, if any (XXX-XXX)

Captures: Links this year's answers to last year's for the same unit, to measure year-to-year progress.

Answer: _____

Part B. Context of the department/course

What this part captures. Describes the size and structure of the unit (students, faculty, calendar, hours) and which of the four technology domains it touches. These numbers are denominators: they turn raw counts (e.g., computers, trained faculty, teaching hours) into comparable ratios.

Answer every item only for the department/course named in Part A. Approximate numbers are acceptable.

B01. Enrollment. Approximate number of students enrolled in this department/course per year

Captures: Size of the student body, used to compute students per computer, per device and per trained teacher.

Answer (students): _____

B02. Faculty headcount. Number of faculty members assigned to this department/course (headcount)

Captures: Teaching staff size, denominator for the share of faculty with AI, informatics, robotics or bioinformatics expertise.

Answer (faculty): _____

B03. Faculty FTE. Full-time equivalent (FTE) of faculty assigned to this department/course

Captures: Teaching capacity adjusted for part-time staff.

Answer (FTE): _____

B04. Academic calendar. Academic calendar used by this department/course

Captures: Calendar type, needed because hours are asked per term and calendars differ between countries.

Country adaptation: see table "Country context", row "Academic calendar".

- ☐ Semester (2 terms per year)
- ☐ Trimester (3 terms per year)
- ☐ Quarter (4 terms per year)
- ☐ Annual system (1 term per year)
- ☐ Modular / block system

B05. Terms per year. Number of academic terms per year (used to convert hours per term into hours per year)

Captures: Converts every "hours per term" answer into hours per year so schools are comparable.

Answer (terms): _____

B06. Course hours. Total contact hours of this course per term (all topics)

Captures: Total course time, to express technology teaching as a share of all teaching time.

Answer (hours per term): _____

B07. Program years. In which program year(s) is this course taught?

Captures: When in the program students meet this content (early exposure vs. clinical years).

- ☐ Year 1
- ☐ Year 2
- ☐ Year 3
- ☐ Year 4
- ☐ Year 5
- ☐ Year 6 or later / internship
- ☐ Graduate / residency year

B08. Course format. Main format of this course

Captures: Teaching setting, because equipment and tool use differ between lecture, lab and clinic courses.

- ☐ Didactic (lectures/seminars)
- ☐ Preclinical / laboratory
- ☐ Clinical
- ☐ Mixed
- ☐ Research

B09. Domains addressed. Which technology domains does this department/course address in teaching, research or care?

Captures: Which of the four ICARB-D domains are present at all, giving a quick profile before the detailed parts.

- ☐ Artificial intelligence
- ☐ Health informatics (EHR, imaging systems, teledentistry, data)
- ☐ Robotics and digital dentistry (CAD/CAM, simulators, robots, navigation)
- ☐ Bioinformatics and omics
- ☐ None of these

B10. Respondent experience. Your own experience with digital technologies (AI, informatics, robotics or bioinformatics) in teaching, research or practice

Captures: Respondent background, used to check whether answers vary with the respondent's own experience.

- ☐ None
- ☐ Less than 1 year
- ☐ 1–3 years
- ☐ More than 3 years

B11. Respondent AI literacy. How would you rate your own AI literacy?

Captures: Self-rated AI literacy of the respondent (not scored as unit maturity).

- ☐ (1) Basic: I know general concepts only
- ☐ (2) Intermediate: I can interpret AI outputs and their limitations
- ☐ (3) Advanced: I can critically appraise AI studies and select tools
- ☐ (4) Expert: I develop, train or validate AI models

Part C. Physical resources: computing, AI tools and data

What this part captures. Pictures the physical and digital resources that every technology domain depends on: computers and GPUs, cloud access, dedicated spaces, software, AI tools actually used by students, datasets, data quality, privacy, connectivity, interoperability and security. It answers "what does this department have today?"

Facilities, equipment, software, data and technical support available to the named department/course.

C01. Computing hardware. Is there hardware designated for AI or data-intensive teaching or research (GPU workstations, high-performance computing, cloud credits) in this department/course? *[domain INF]*

Captures: Whether the unit has its own computing power for AI and data work, or depends on personal equipment.

- ☐ (1) None
- ☐ (2) Only individual faculty members' personal equipment
- ☐ (3) Shared equipment, insufficient for demand
- ☐ (4) Dedicated high-performance computing, regularly updated and accessible to students
- ☐ (-1) Don't know / Not applicable

C02. GPU workstations. Number of GPU-equipped workstations or servers available to this department/course

Captures: Hard count of AI-capable machines, comparable across schools and over years.

Answer (units): _____

C03. Cloud computing. Access to cloud computing (institutional cloud accounts, research credits, university HPC cluster) *[domain INF]*

Captures: Access to scalable computing beyond local machines.

- ☐ (1) No access
- ☐ (2) Individual/ad hoc access paid by faculty
- ☐ (3) Institutional access on request, limited quota
- ☐ (4) Routine institutional access with support
- ☐ (-1) Don't know / Not applicable

C04. Student devices ratio. Number of students per institutionally provided computer during hands-on digital sessions (e.g., 1 = one computer per student)

Captures: Whether students can actually practise hands-on, or only watch.

Answer (students per computer): _____

C05. Dedicated digital space. Is there a dedicated physical space for digital dentistry, informatics or AI teaching/research (lab, innovation hub)? *[domain INF]*

Captures: Existence of a physical home for digital work, a strong sign of institutional commitment.

- ☐ (1) None
- ☐ (2) Shared general computer room
- ☐ (3) Dedicated digital dentistry lab without AI focus
- ☐ (4) Dedicated AI/digital dentistry lab with staff and scheduled student access
- ☐ (-1) Don't know / Not applicable

C06. Software licences. Which data-science and AI software is available to students and faculty of this department/course?

Captures: The software toolbox available, from programming languages to annotation and commercial AI.

- ☐ Python / Jupyter / Google Colab
- ☐ R / RStudio
- ☐ MATLAB
- ☐ No-code machine learning (e.g., Orange, KNIME, Teachable Machine)
- ☐ Image annotation tools (e.g., CVAT, Label Studio, 3D Slicer)
- ☐ Statistical packages (SPSS, Stata, SAS)
- ☐ Commercial dental AI platforms

- ☐ Institutionally licensed generative AI (e.g., Copilot, ChatGPT Edu, Gemini)
- ☐ None

Other, specify: _____

C07. AI-enabled equipment. Which AI-enabled equipment or systems are in use in this department/course?

Captures: Where AI is already embedded in equipment, often without being taught explicitly.

- ☐ Digital radiography with AI analysis
- ☐ CBCT with AI segmentation
- ☐ Intraoral scanners with AI features
- ☐ CAD/CAM or 3D printing with AI design
- ☐ Haptic / simulation units with AI feedback
- ☐ Electronic health record with AI modules
- ☐ Learning management system with AI features
- ☐ AI tutor / chatbot for students
- ☐ None

Other, specify: _____

C08. AI tools used by students. List each AI tool students actually use in this department/course. Leave rows empty if not applicable.

Captures: An inventory of real AI tools: what they do, whether they are approved, how deeply students use them and since when. Pooled across schools it becomes a map of dental AI adoption.

#	Tool / product name	Main task	Regulatory status	Student use	Part of graded work?	Year introduced
1						
2						
3						
4						
5						
6						

Main task codes: caries = Caries detection on radiographs; perio = Periodontal bone-loss assessment; periapical = Periapical lesion detection; cbct = CBCT segmentation / 3D analysis; ceph = Cephalometric landmarking / orthodontic analysis; implant = Implant planning / guided surgery; cad = CAD/CAM restoration design; ios = Intraoral-scan analysis / smile design; lesion = Oral mucosal lesion / oral cancer screening; charting = Charting, clinical notes, ambient scribe; patient = Patient communication / education; admin = Scheduling, billing, administration; sim = Preclinical simulation feedback / preparation grading; llm = General-purpose LLM / chatbot; other = Other

Regulatory status codes: 1 = Cleared/approved by the national regulator for this use; 2 = Foreign approval only (e.g., CE, FDA); 3 = Commercial, not cleared; 4 = Research / in-house prototype; -1 = Don't know

Student use codes: 1 = Students observe faculty use; 2 = Preclinical / simulation only; 3 = Students operate under supervision on patients; 4 = Students operate independently in patient care

Part of graded work? codes: 1 = Yes; 0 = No

C09. Number of AI tools. Overall use of AI tools by students in this department/course *[domain INF]*

Captures: Summary of AI tool adoption for scoring.

- ☐ (1) No AI tool is used
- ☐ (2) 1 AI tool used
- ☐ (3) 2–3 AI tools used

- ☐ (4) 4 or more AI tools used and integrated into graded coursework
- ☐ (-1) Don't know / Not applicable

C10. De-identified dataset. Is there a standardized, de-identified oral-health dataset (radiographs, intraoral scans, treatment records) that students or faculty can access for AI training or research? *[domain DAT]*

Captures: Availability of data to train and test AI, the key ingredient for the consortium's open AI model.

- ☐ (1) No dataset accessible
- ☐ (2) Dataset in planning or pilot stage
- ☐ (3) Dataset exists but access is restricted and scale is limited
- ☐ (4) Dataset exists with clear access protocols and is used in the course
- ☐ (-1) Don't know / Not applicable

C11. Dataset modalities. Which data modalities are available in that dataset?

Captures: Which data types exist, to plan which parts of a shared AI model each school could support.

- ☐ Periapical / bitewing radiographs
- ☐ Panoramic radiographs
- ☐ Cephalometric radiographs
- ☐ CBCT
- ☐ Intraoral scans (3D)
- ☐ Clinical photographs
- ☐ Histopathology images
- ☐ Structured treatment / EHR records
- ☐ Free-text clinical notes
- ☐ No dataset

C12. Dataset size. Approximate number of de-identified images or records in that dataset (enter 0 if none)

Captures: Scale of existing data.

Answer (images/records): _____

C13. Annotation capacity. Capacity to label/annotate data for AI (calibrated annotators, annotation protocol, inter-rater agreement) *[domain DAT]*

Captures: Ability to produce reliable labels, which determines whether data can be used to train trustworthy AI.

- ☐ (1) No capacity
- ☐ (2) Ad hoc annotation by individual researchers
- ☐ (3) Annotation protocol exists, limited calibration
- ☐ (4) Calibrated annotators with documented protocol and agreement statistics
- ☐ (-1) Don't know / Not applicable

C14. Data quality. What measures ensure the accuracy, completeness and timeliness of data used in this department/course? *[domain DAT]*

Captures: Data quality assurance (PAHO/WHO readiness toolkit item).

- ☐ (1) No measures applied
- ☐ (2) Ad hoc checks only
- ☐ (3) Some measures exist but are not comprehensive
- ☐ (4) Systematic quality assurance with documented protocols
- ☐ (-1) Don't know / Not applicable

C15. Data representativeness. Is the representativeness of the data assessed (age, sex, ethnicity, socioeconomic group, rural/urban, device type)? *[domain DAT]*

Captures: Whether datasets reflect the patients the AI will serve (PAHO bias-free AI guidance).

- ☐ (1) Never assessed
- ☐ (2) Discussed informally
- ☐ (3) Assessed for some projects
- ☐ (4) Systematically assessed and reported
- ☐ (-1) Don't know / Not applicable

C16. Privacy and security. What privacy and security systems protect patient data used in this department/course? *[domain DAT]*

Captures: Protection of patient data, read against each country's privacy law.

Country adaptation: see table "Country context", row "Privacy law".

- ☐ (1) No systems
- ☐ (2) Basic password protection only
- ☐ (3) Systems exist but are inconsistent
- ☐ (4) Comprehensive systems with encryption, audit trails and a named person responsible
- ☐ (-1) Don't know / Not applicable

C17. De-identification. Are there documented procedures for anonymization / de-identification of patient data before use in teaching or research? *[domain DAT]*

Captures: Readiness to share data safely, a precondition for joint AI development.

- ☐ (1) No procedures
- ☐ (2) Informal practice only
- ☐ (3) Written procedures, inconsistently applied
- ☐ (4) Written procedures, routinely applied and audited
- ☐ (-1) Don't know / Not applicable

C18. Secure research environment. Is a secure environment available for working with identifiable or sensitive data (trusted research environment, approved server, data enclave)? *[domain DAT]*

Captures: Whether sensitive data can be analysed without leaving a protected environment.

- ☐ (1) Not available
- ☐ (2) Planned
- ☐ (3) Available on request with delays
- ☐ (4) Available and routinely used
- ☐ (-1) Don't know / Not applicable

C19. Connectivity and IT support. Does this department/course have reliable broadband and technical support for the digital tools it uses? *[domain INF]*

Captures: Basic connectivity and support, without which equipment goes unused.

- ☐ (1) Neither reliable connectivity nor specific support
- ☐ (2) Basic IT support exists but does not cover these tools
- ☐ (3) Partial support (software installation only)
- ☐ (4) Dedicated educational-technology support assigned to the department/course
- ☐ (-1) Don't know / Not applicable

C20. Interoperability. Can the digital tools used exchange data with existing systems (EHR, imaging/PACS, learning management system) using standards such as DICOM, HL7 FHIR or LTI? *[domain INF]*

Captures: Whether systems talk to each other, a core health-informatics capability.

- ☐ (1) No interoperability
- ☐ (2) Some standards under development
- ☐ (3) Standards exist but implementation is inconsistent
- ☐ (4) Well-established standards enabling seamless integration
- ☐ (-1) Don't know / Not applicable

C21. Cybersecurity. Are cybersecurity measures applied to digital tools and data (risk assessment, access control, vendor security review)? *[domain INF]*

Captures: Security of systems that hold patient data.

- ☐ (1) None
- ☐ (2) Basic institutional measures only
- ☐ (3) Measures exist but not specific to these tools
- ☐ (4) Specific security review before adoption and periodic audit
- ☐ (-1) Don't know / Not applicable

C22. Equitable student access. Do all students in this course have equal access to the tools and computing needed (including outside class hours)? *[domain INF]*

Captures: Equity of access among students.

- ☐ (1) No — access depends on personal devices/subscriptions
- ☐ (2) Limited institutional access
- ☐ (3) Access during scheduled sessions only
- ☐ (4) Equal access during and outside class hours
- ☐ (-1) Don't know / Not applicable

Part D. Human resources and expertise (AI and data science)

What this part captures. Pictures the people: how many faculty can teach AI, who does AI research, what training they received, which partners and technical staff support them, and what holds them back. Equipment without trained people does not change teaching; this part measures that capacity.

Faculty and staff assigned to the named department/course.

D01. Faculty able to teach AI. What proportion of faculty can independently teach AI content (machine-learning principles, interpreting AI outputs, critical appraisal of AI literature)? *[domain HUM]*

Captures: Teaching capacity for AI as a share of all faculty.

- ☐ (1) 0%
- ☐ (2) 1–10%
- ☐ (3) 11–25%
- ☐ (4) More than 25%
- ☐ (-1) Don't know / Not applicable

D02. Faculty teaching AI (n). Number of faculty who currently teach any AI content in this department/course

Captures: Exact count, comparable across years.

Answer (faculty): _____

D03. Faculty with formal AI degree. Number of faculty with a formal qualification in AI, computer science, data science or biomedical informatics (degree or university certificate)

Captures: Deep expertise in the unit, beyond self-taught knowledge.

Answer (faculty): _____

D04. AI-primary researchers. How many faculty have AI in dentistry or dental education as their primary research area? *[domain HUM]*

Captures: Research leadership in AI, often the engine of curricular change.

- ☐ (1) None
- ☐ (2) 1
- ☐ (3) 2–3
- ☐ (4) 4 or more
- ☐ (-1) Don't know / Not applicable

D05. AI publications (3 y). Number of peer-reviewed AI-related publications by faculty of this department/course in the past 3 years

Captures: Research output, a verifiable indicator of AI activity.

Answer (publications): _____

D06. AI graduate trainees. Number of graduate students, residents or postdocs currently working on AI projects in this department/course

Captures: The next generation of AI researchers being trained.

Answer (trainees): _____

D07. Faculty AI training. What formal AI training have faculty received in the past 24 months? *[domain HUM]*

Captures: Institutional investment in faculty development.

- ☐ (1) No training received
- ☐ (2) Voluntary, sporadic sessions only
- ☐ (3) Organized training, but low or inconsistent attendance
- ☐ (4) Systematic training with institutional incentives and tracking
- ☐ (-1) Don't know / Not applicable

D08. Training hours per faculty. Average hours of AI training per faculty member in the past 12 months

Captures: Intensity of faculty training.

Answer (hours): _____

D09. Training formats. Formats of AI training available to faculty

Captures: How faculty learn, to design consortium training that fills gaps.

- ☐ Institutional workshops / seminars
- ☐ Online courses (MOOCs, e.g., Coursera, edX)
- ☐ University certificate programs
- ☐ Conference sessions (IADR, ADEA, ADEE, SBPqO, IDA, CSA)
- ☐ Peer mentoring / communities of practice
- ☐ Vendor training
- ☐ Sponsored degree programs
- ☐ None

Other, specify: _____

D10. Interdisciplinary partners. Name each external unit this department/course collaborates with on AI, informatics, robotics or bioinformatics (e.g., Department of Computer Science, Faculty of Engineering, a company).

Captures: The collaboration network, showing who dental schools rely on for technical expertise.

#	Partner unit	Type
1		
2		
3		

Type codes: 1 = Same university — computer science / engineering; 2 = Same university — medicine / public health / biology / other; 3 = Other university (national); 4 = Other university (international); 5 = Industry / company; 6 = Government / hospital / other

D11. Partnership depth. Depth of interdisciplinary collaboration *[domain HUM]*

Captures: How formal and durable those partnerships are.

- ☐ (1) No partners
- ☐ (2) Informal individual collaboration only
- ☐ (3) Formal project-based collaboration
- ☐ (4) Long-term institutional partnership with joint funding or appointments
- ☐ (-1) Don't know / Not applicable

D12. Technical staff. Access to dedicated technical personnel (data scientists, AI engineers, statisticians, research software engineers) *[domain HUM]*

Captures: Technical support available to faculty and students.

- ☐ (1) No access
- ☐ (2) Shared across the school, high demand
- ☐ (3) Adequate access for most needs
- ☐ (4) Dedicated technical staff embedded in the department/course
- ☐ (-1) Don't know / Not applicable

D13. Technical staff FTE. Full-time equivalent of technical data/AI staff available to this department/course

Captures: Exact technical staffing, comparable over years.

Answer (FTE): _____

D14. Hiring plans. Is recruitment of faculty or staff with AI, informatics, robotics or bioinformatics expertise planned? *[domain HUM]*

Captures: Forward-looking investment in people.

- ☐ (1) Not planned
- ☐ (2) Discussed, no approved position
- ☐ (3) Position approved or recruitment underway
- ☐ (4) Expertise already recruited and further positions planned
- ☐ (-1) Don't know / Not applicable

D15. Faculty openness. Overall attitude of faculty in this department/course toward integrating these technologies *[domain HUM]*

Captures: Cultural readiness for change.

- ☐ (1) Mostly resistant
- ☐ (2) Mixed / indifferent

- ☐ (3) Mostly supportive
- ☐ (4) Strongly supportive with active champions
- ☐ (-1) Don't know / Not applicable

D16. Faculty competency barriers. Most significant barriers to faculty competency (select up to two)

Captures: The main obstacles, so consortium support can target them.

Select up to 2.

- ☐ Lack of faculty time
- ☐ Lack of funding for training
- ☐ Lack of faculty interest
- ☐ Lack of qualified trainers
- ☐ Competing curricular priorities
- ☐ No recognition in promotion/tenure
- ☐ Training not available in local language

Other, specify: _____

Part E. AI curriculum and teaching

What this part captures. Measures what students are actually taught about AI and for how long: hours per term by format and by 16 topics (including programming and formal model development), named undergraduate and graduate courses, assessment, materials and planned changes. Hours are converted to hours per year (with B05) to build an international benchmark.

Refers to the named course (Part A, item A07). Hours: include lectures, hands-on sessions and assignments; exclude generic literature searching and routine use of clinical software.

E01. Dedicated AI instruction. Does the course include dedicated AI instruction (not a passing mention)? *[domain CUR]*

Captures: Whether AI is taught deliberately, not only mentioned.

- ☐ (1) None
- ☐ (2) Mentioned in 1–2 lectures only
- ☐ (3) 1 dedicated module or class session (≤4 hours)
- ☐ (4) 1 or more dedicated AI courses, or systematic integration across sessions
- ☐ (-1) Don't know / Not applicable

E02. AI hours by format. Hours of AI-specific instruction a student receives in this course per term, by format (enter 0 if none)

Captures: Total AI teaching time and how much of it is hands-on rather than passive.

Topic / format	Hours (hours per term)
Lectures / seminars	
Hands-on / computer lab	
Clinical / preclinical use with supervision	
Assignments / guided self-study	

E03. AI hours by topic. Hours per term devoted to each AI topic in this course (enter 0 if not covered; a session may count toward more than one topic)

Captures: The content profile of AI teaching across 16 topics, from fundamentals to programming, validation, bias and regulation.

Topic / format	Hours (hours per term)
AI fundamentals and terminology	
Machine learning / deep learning principles	
Data science and statistics for AI	
Programming (Python, R)	
Model development and training	
Model validation and performance metrics (sensitivity, AUC, calibration)	
Bias, fairness and subgroup performance	
Explainability / interpretability	
Clinical interpretation of AI outputs and AI errors	
AI in dental imaging (radiographs, CBCT)	
AI in treatment planning, CAD/CAM, orthodontics, implants	
Generative AI, large language models and prompt design	
Ethics, privacy, informed consent, accountability	
Regulation, standards and approval of AI devices	
Critical appraisal of AI literature and reporting guidelines	
AI as a research tool (literature, analysis, writing)	

E04. Required or elective. Is the AI content of this course required or elective?

Captures: Whether every graduate is exposed, or only volunteers.

- ☐ No AI content
- ☐ Elective / optional
- ☐ Required for some students / tracks
- ☐ Required for all students

E05. AI credits. Academic credits awarded for AI content (local credit units; 0 if none)

Captures: Formal weight of AI in the curriculum.

Answer (credits): _____

E06. Technical depth. Which best describes the technical depth of AI instruction? *[domain CUR]*

Captures: Depth of AI teaching, from awareness to building models.

- ☐ (1) Conceptual awareness only
- ☐ (2) Clinical interpretation of AI outputs
- ☐ (3) Hands-on operation of existing AI tools under guidance
- ☐ (4) Includes programming or formal AI development
- ☐ (-1) Don't know / Not applicable

E07. Programming / AI development. Does the course cover programming (Python/R) or formal AI development (model training, validation, bias assessment)? *[domain CUR]*

Captures: Whether students gain the skills needed to contribute to building AI.

- ☐ (1) None
- ☐ (2) Instructor demonstration only
- ☐ (3) Hands-on exercises under guidance
- ☐ (4) Students independently complete a small AI project (e.g., train and validate a classifier)
- ☐ (-1) Don't know / Not applicable

E08. Development platforms. Platforms used for programming or model development

Captures: The tools used for hands-on development.

- ☐ Python / Jupyter / Colab
- ☐ R
- ☐ No-code tool (Orange, KNIME, Teachable Machine)
- ☐ Cloud AutoML (Google, Azure, AWS)
- ☐ Vendor research platform
- ☐ Not applicable

Other, specify: _____

E09. Programming hours. Hours per term devoted to programming or model development

Captures: Time spent on programming and formal AI development specifically.

Answer (hours per term): _____

E10. Student AI projects. Number of student AI projects (coursework, theses, capstones) completed in the past academic year

Captures: Concrete student output in AI.

Answer (projects): _____

E11. Undergraduate AI courses. Name the undergraduate course(s) in this department that are dedicated to AI or contain a dedicated AI module

Captures: The named undergraduate AI courses and their hours, for the transparency list in publications.

#	Course code and title	AI hours per term	Status
1			
2			
3			

Status codes: 1 = Elective; 2 = Required

E12. Graduate / residency AI courses. Name the graduate or residency course(s) in this department that include AI content

Captures: The named graduate and residency AI courses and their hours.

#	Course code and title	AI hours per term	Status
1			
2			
3			

Status codes: 1 = Elective; 2 = Required

E13. Graduate AI offering. Graduate-level AI offering in this department *[domain CUR]*

Captures: Maturity of AI at graduate level.

- ☐ (1) No graduate AI course
- ☐ (2) Elective only
- ☐ (3) Required course exists
- ☐ (4) Required course plus a formal AI research project requirement
- ☐ (-1) Don't know / Not applicable

E14. AI theses (3 y). Number of master's/doctoral theses or residency projects on AI defended in this department in the past 3 years

Captures: Graduate research output in AI.

Answer (theses): _____

E15. Clinical AI training. Are students trained in clinical application and critical interpretation of AI outputs (reading AI-assisted reports, identifying AI errors, understanding limitations)? *[domain CUR]*

Captures: Preparation for safe use of AI with patients.

- ☐ (1) None
- ☐ (2) Informal discussion only
- ☐ (3) 1 dedicated session
- ☐ (4) Integrated into clinical rotations with formal assessment
- ☐ (-1) Don't know / Not applicable

E16. Assessment of AI competency. How is student AI competency assessed? *[domain CUR]*

Captures: Whether AI learning is verified, not just delivered.

- ☐ (1) Not assessed
- ☐ (2) Written exam only
- ☐ (3) Separate AI assignment or project
- ☐ (4) Competency-based assessment (OSCE station, portfolio, practical demonstration)
- ☐ (-1) Don't know / Not applicable

E17. Teaching methods. Teaching methods used for AI content

Captures: How AI is taught, to identify practices worth sharing across the consortium.

- ☐ Lectures
- ☐ Flipped classroom
- ☐ Case-based learning
- ☐ Problem-based learning
- ☐ Journal club / critical appraisal
- ☐ Computer lab / coding exercises
- ☐ Hackathon / competition
- ☐ Supervised clinical use of AI tools
- ☐ Simulation with AI feedback
- ☐ AI tutor / chatbot-assisted learning

Other, specify: _____

E18. Educational materials. Dedicated AI educational materials (textbooks, online modules, case libraries) *[domain CUR]*

Captures: Quality of teaching resources.

- ☐ (1) None used

- ☐ (2) Scattered resources, not formally organized
- ☐ (3) Some dedicated materials used
- ☐ (4) Comprehensive, curated collection integrated into the course
- ☐ (-1) Don't know / Not applicable

E19. Materials used. Name the main AI educational materials used

Captures: Specific materials, to build a shared open library.

#	Material (title / URL)
1	
2	
3	

E20. Competency framework. Is AI teaching aligned with a competency framework (e.g., IADR/ITU-WHO core curriculum for dental AI, national curriculum guidelines)? *[domain CUR]*

Captures: Alignment with international and national curriculum guidance.

Country adaptation: see table "Country context", row "Curriculum / accreditation body".

- ☐ (1) No framework
- ☐ (2) Aware of frameworks, not used
- ☐ (3) Partially aligned
- ☐ (4) Explicitly mapped to a framework with defined learning outcomes
- ☐ (-1) Don't know / Not applicable

E21. AI as a teaching tool. Is AI used as a tool to deliver teaching (adaptive learning, automated feedback on preparations, AI-generated cases, virtual patients)? *[domain CUR]*

Captures: AI used to teach, not only taught as a topic.

- ☐ (1) Not used
- ☐ (2) Experimented by individual faculty
- ☐ (3) Used regularly in some activities
- ☐ (4) Systematically used and evaluated
- ☐ (-1) Don't know / Not applicable

E22. CPD AI offerings. Number of continuing-education (CPD) activities on AI offered by this department in the past 12 months

Captures: Reach of the department beyond its own students, to practising dentists.

Answer (activities): _____

E23. Student preparedness. How prepared and interested in AI are students in this course? *[domain CUR]*

Captures: Student readiness as perceived by faculty.

- ☐ (1) Low interest and preparedness
- ☐ (2) Interest but low preparedness
- ☐ (3) Interest and moderate preparedness
- ☐ (4) High interest and preparedness
- ☐ (-1) Don't know / Not applicable

E24. Planned curriculum change. Is an increase of AI content planned for the next 2 years? *[domain CUR]*

Captures: Momentum: where the curriculum is heading.

- ☐ (1) No plans

- □ (2) Under informal discussion
- □ (3) Approved, not yet implemented
- □ (4) Being implemented now
- □ (-1) Don't know / Not applicable

Part F. Generative AI in teaching and learning

What this part captures. Captures how the unit governs and teaches generative AI: policies, permitted uses, prompt-design and verification skills, and safeguards (adapted from PAHO "AI prompt design" and AI-GUARD Tier 1). Generative AI spread faster than any earlier technology, so this part is tracked closely year to year.

Generative AI includes chatbots and large language models such as ChatGPT, Gemini, Copilot, DeepSeek, Qwen or Ernie Bot.

F01. Generative AI policy. Which policy governs student and faculty use of generative AI in this department/course? *[domain GEN]*

Captures: Maturity of generative-AI rules.

- □ (1) No policy
- □ (2) General academic-integrity policy only, not AI-specific
- □ (3) Interim guidance issued
- □ (4) Formal, regularly updated policy with training
- □ (-1) Don't know / Not applicable

F02. Policy name. Name of the policy document (and level: university / school / department / course)

Captures: The actual document, so policies can be compared and shared.

Answer: _____

F03. Permitted student uses. Which student uses of generative AI are permitted in this course?

Captures: What students may actually do with generative AI.

- □ Studying / explanations
- □ Writing support with disclosure
- □ Practising clinical cases
- □ Coding assistance
- □ Literature search / research support
- □ Use during assessments
- □ All use is prohibited
- □ Not defined

Other, specify: _____

F04. Prompt design teaching. Are students taught how to design effective prompts (context, task, format, audience, constraints)? *[domain GEN]*

Captures: Prompt design as a core digital skill (PAHO).

- □ (1) Not taught
- □ (2) Mentioned informally
- □ (3) 1 dedicated session
- □ (4) Integrated practice with feedback
- □ (-1) Don't know / Not applicable

F05. Verification of outputs. Are students taught to verify generative-AI outputs (hallucinations, fabricated references, outdated guidance)? *[domain GEN]*

Captures: Critical checking of AI output, essential for patient safety.

- ☐ (1) Not taught
- ☐ (2) Mentioned informally
- ☐ (3) 1 dedicated session
- ☐ (4) Integrated practice with assessment
- ☐ (-1) Don't know / Not applicable

F06. Generative AI safeguards. Which safeguards are in place for generative AI use in this department/course? (adapted from PAHO AI-GUARD, Tier 1)

Captures: A safeguards checklist scored Yes = 2, Partial = 1, No = 0 into a generative-AI safeguards index.

Item	Yes	Partial	No	Don't know
Rule that no identifiable patient data is entered into public AI tools	☐	☐	☐	☐
An institutionally licensed, privacy-protected generative AI tool is available	☐	☐	☐	☐
Students must disclose AI use in assignments	☐	☐	☐	☐
Human verification required before AI output is used in patient care	☐	☐	☐	☐
Process to report misuse or AI errors	☐	☐	☐	☐
Staff and students trained on limitations and bias of generative AI	☐	☐	☐	☐

Scoring: Yes = 2, Partial = 1, No = 0.

F07. Faculty GenAI use. How often do faculty use generative AI to prepare teaching materials or assessments?

Captures: Faculty adoption of generative AI in their own work.

- ☐ Never
- ☐ Occasionally
- ☐ Monthly
- ☐ Weekly or more
- ☐ Don't know

F08. AI-detection software. Is AI-text detection software used to assess student work?

Captures: Reliance on detection tools, which have known accuracy problems.

- ☐ Yes, routinely
- ☐ Yes, occasionally
- ☐ No
- ☐ Don't know

Part G. Health informatics

What this part captures. Measures the digital backbone of the school clinic and how informatics is taught: electronic records, imaging archives, coding, teledentistry, analytics and research data access, plus teaching hours and staff. Health informatics produces the structured data that AI and research depend on.

Health informatics covers electronic health records, imaging systems, coding and terminologies, teledentistry, data analytics and geographic information systems.

G01. Electronic health record. Which best describes the patient record system used in the clinics of this department? *[domain HIN]*

Captures: Digital maturity of patient records, the main source of clinical data.

- ☐ (1) Paper records
- ☐ (2) Electronic scheduling/billing only; clinical notes on paper
- ☐ (3) Electronic clinical records, mostly free text
- ☐ (4) Integrated electronic record with structured data, imaging and decision support
- ☐ (-1) Don't know / Not applicable

G02. EHR system name. Name the electronic record and imaging (PACS) systems used (e.g., axiUm, Dentrix, MiPACS)

Captures: The actual systems, to study interoperability and shared data formats across schools.

Answer: _____

G03. Coding and terminologies. Which coding systems are used to record diagnoses and procedures?

Captures: Use of standard codes, which makes data comparable and usable for AI.

- ☐ ICD-10 / ICD-11
- ☐ SNODENT / SNOMED CT
- ☐ Procedure codes (CDT, TUSS or national equivalent)
- ☐ Free text only / none

Other, specify: _____

G04. Digital imaging archive. How are dental images stored and shared? *[domain HIN]*

Captures: Whether images are centrally available, a precondition for imaging AI.

- ☐ (1) Film or images not archived centrally
- ☐ (2) Digital images stored per device or computer
- ☐ (3) Central archive (PACS) for some modalities
- ☐ (4) Enterprise PACS integrated with the health record via DICOM
- ☐ (-1) Don't know / Not applicable

G05. Teledentistry. Use of teledentistry (remote consultation, triage, monitoring) in this department *[domain HIN]*

Captures: Remote care capacity and student exposure to it.

- ☐ (1) None
- ☐ (2) Ad hoc use by individuals
- ☐ (3) Structured program in some services
- ☐ (4) Routine teledentistry with student involvement
- ☐ (-1) Don't know / Not applicable

G06. Data analytics. Are clinic and teaching data analysed with dashboards or routine reports? *[domain HIN]*

Captures: Use of data for decisions, a sign of informatics maturity.

- ☐ (1) Never
- ☐ (2) Occasional manual reports
- ☐ (3) Regular reports for some indicators
- ☐ (4) Routine dashboards used for clinical and teaching quality decisions
- ☐ (-1) Don't know / Not applicable

G07. Research access to clinical data. Can researchers access clinic data (de-identified) for research? *[domain HIN]*

Captures: Ability to turn routine care into research and AI datasets.

- ☐ (1) Not possible
- ☐ (2) Ad hoc extracts by IT
- ☐ (3) Formal request process
- ☐ (4) Curated research data warehouse with governance
- ☐ (-1) Don't know / Not applicable

G08. Public-health informatics / GIS. Use of population health informatics (geographic information systems, surveillance data, epidemiological databases) in teaching or research *[domain HIN]*

Captures: Use of population-level and geographic data in oral public health.

- ☐ (1) Never
- ☐ (2) Occasionally
- ☐ (3) In some projects or courses
- ☐ (4) Routinely
- ☐ (-1) Don't know / Not applicable

G09. Health informatics hours by topic. Hours per term of health-informatics teaching in this course, by topic (enter 0 if none)

Captures: Time spent teaching health informatics, by topic, converted to hours per year.

Topic / format	Hours (hours per term)
Electronic records and documentation quality	
Coding, terminologies and interoperability standards	
Teledentistry and digital communication	
Data analytics, epidemiology and GIS	
Privacy, cybersecurity and data protection law	

G10. Informatics teaching depth. Depth of health-informatics teaching *[domain HIN]*

Captures: Whether informatics is a real part of the curriculum.

- ☐ (1) None
- ☐ (2) Mentioned in lectures
- ☐ (3) Dedicated module
- ☐ (4) Dedicated course with practical assessment
- ☐ (-1) Don't know / Not applicable

G11. Faculty with informatics training. Number of faculty with formal training in health informatics or GIS

Captures: Human capacity in health informatics.

Answer (faculty): _____

G12. Clinical informatics staff. Support from clinical informatics staff (EHR analysts, imaging/PACS administrators) *[domain HIN]*

Captures: Technical staff who keep clinical systems running and data usable.

- ☐ (1) None
- ☐ (2) Central university IT only
- ☐ (3) Shared clinical informatics staff

- ☐ (4) Dedicated clinical informatics team for the school
- ☐ (-1) Don't know / Not applicable

Part H. Robotics and digital dentistry

What this part captures. Takes an inventory of the physical equipment for robotics and digital dentistry (how many units of each), how students access it, how digital workflows are used, who can teach and maintain it, teaching hours and research. Counted every year, it shows how equipment and skills spread across schools.

Includes robot-assisted surgery, haptic and virtual-reality simulators, navigation, scanners, CAD/CAM milling and 3D printing.

H01. Equipment inventory. Number of units available to this department/course (enter 0 if none)

Captures: A hard count of equipment, comparable across schools and years and convertible to students per unit.

Item	Number (units available)
Robot-assisted implant surgery systems	
Other surgical, endodontic or automated robotic systems	
Dynamic navigation systems for guided surgery	
Haptic or virtual-reality dental simulators	
Conventional simulation units (phantom heads)	
Intraoral scanners	
CBCT units	
Chairside or lab milling units (CAD/CAM)	
3D printers	
Augmented/virtual-reality headsets	

H02. Equipment age. Average age of the main digital and robotic equipment

Captures: How up to date the equipment is.

- ☐ Less than 3 years
- ☐ 3–5 years
- ☐ 6–10 years
- ☐ More than 10 years
- ☐ Don't know

H03. Student access to robotics. Student access to robotic, haptic or navigation systems *[domain ROB]*

Captures: Whether students actually use the equipment.

- ☐ (1) No access
- ☐ (2) Demonstration only
- ☐ (3) Supervised hands-on practice in preclinic
- ☐ (4) Regular hands-on practice including supervised clinical cases
- ☐ (-1) Don't know / Not applicable

H04. Digital workflow. Use of a full digital workflow (scan, design, manufacture) in teaching and clinics *[domain ROB]*

Captures: Integration of digital dentistry into everyday care and teaching.

- ☐ (1) Analog only
- ☐ (2) Partial digital steps
- ☐ (3) Full digital workflow in some clinics
- ☐ (4) Full digital workflow routine across clinics and teaching
- ☐ (-1) Don't know / Not applicable

H05. Robotics/digital hours by topic. Hours per term of robotics and digital-dentistry teaching in this course, by topic (enter 0 if none)

Captures: Time spent teaching robotics and digital dentistry, by topic.

Topic / format	Hours (hours per term)
Digital workflow, CAD/CAM and 3D printing	
Haptic / virtual-reality simulation	
Robot-assisted surgery (principles, demonstration, practice)	
Navigation and guided surgery	
Engineering, automation and maintenance basics	

H06. Faculty trained in robotics. Number of faculty trained to operate or teach robotic, navigation or haptic systems

Captures: Teaching capacity for robotics and digital dentistry.

Answer (faculty): _____

H07. Technical support FTE. Full-time equivalent of technicians or engineers supporting digital and robotic equipment

Captures: Staff who keep the equipment working.

Answer (FTE): _____

H08. Maintenance and replacement. Maintenance and replacement of digital/robotic equipment *[domain ROB]*

Captures: Whether investment in equipment is sustainable.

- ☐ (1) No plan
- ☐ (2) Ad hoc repairs
- ☐ (3) Service contracts for some equipment
- ☐ (4) Full maintenance and replacement plan with budget
- ☐ (-1) Don't know / Not applicable

H09. Safety and credentialing. Training and credentialing protocols before robotic or navigation systems are used on patients *[domain ROB]*

Captures: Patient safety when new technology is used in the clinic.

- ☐ (1) None
- ☐ (2) Informal supervision
- ☐ (3) Written protocol, not always applied
- ☐ (4) Formal credentialing with documented training and supervision
- ☐ (-1) Don't know / Not applicable

H10. Robotics research. Robotics and digital-dentistry research in this department *[domain ROB]*

Captures: Research activity that drives innovation.

- ☐ (1) None
- ☐ (2) Individual projects
- ☐ (3) Funded projects
- ☐ (4) Established research group with engineering partners
- ☐ (-1) Don't know / Not applicable

Part I. Bioinformatics and omics

What this part captures. Pictures laboratory and computing resources for omics research (sequencing, biobanks, pipelines), the people with bioinformatics skills, teaching hours and data-sharing practices. It shows which schools can contribute biological data to future multi-modal AI that links images, records and biology.

Bioinformatics covers the computational analysis of biological data such as the oral microbiome, genomes, gene expression, proteins and metabolites.

I01. Laboratory resources. Which resources are available to this department/course?

Captures: The physical infrastructure for generating and analysing biological data.

- ☐ Molecular biology laboratory
- ☐ DNA/RNA sequencer in the department or school
- ☐ University sequencing / genomics core facility
- ☐ Biobank of oral samples (saliva, plaque, tissue)
- ☐ Mass spectrometry (proteomics / metabolomics)
- ☐ High-performance computing for bioinformatics
- ☐ None

Other, specify: _____

I02. Biobank and sample governance. Status of biological sample collection and storage *[domain BIO]*

Captures: Ability to run omics studies and link biology to clinical data.

- ☐ (1) None
- ☐ (2) Samples collected per project only
- ☐ (3) Biobank with consent protocol
- ☐ (4) Biobank with linked clinical data, governance and use by several projects
- ☐ (-1) Don't know / Not applicable

I03. Research areas. Bioinformatics and omics research areas active in this department

Captures: Which omics fields the department works in, to match partners across the consortium.

- ☐ Oral microbiome
- ☐ Host genomics / genetic association studies
- ☐ Transcriptomics
- ☐ Proteomics / metabolomics (e.g., saliva)
- ☐ Epigenetics
- ☐ Craniofacial and developmental genetics
- ☐ Antimicrobial resistance
- ☐ None

Other, specify: _____

I04. Software and pipelines. Bioinformatics software and resources used

Captures: Computational tools in use, and whether analyses are reproducible.

- ☐ QIIME 2 / mothur (microbiome)
- ☐ R / Bioconductor
- ☐ Galaxy
- ☐ Python (Biopython, scikit-bio)
- ☐ Workflow managers (Nextflow, Snakemake)
- ☐ Public databases (NCBI, HOMD, UniProt)
- ☐ None

Other, specify: _____

I05. Faculty with bioinformatics expertise. Number of faculty with bioinformatics or computational-biology expertise

Captures: Human capacity in bioinformatics.

Answer (faculty): _____

I06. Bioinformatics support. Access to bioinformatics support *[domain BIO]*

Captures: Whether biological data can be analysed in-house.

- ☐ (1) None
- ☐ (2) External collaborators only
- ☐ (3) Shared university core facility
- ☐ (4) Dedicated bioinformatician in the department
- ☐ (-1) Don't know / Not applicable

I07. Bioinformatics hours by topic. Hours per term of bioinformatics teaching in this course, by topic (enter 0 if none)

Captures: Time spent teaching bioinformatics, by topic.

Topic / format	Hours (hours per term)
Molecular biology and genetics foundations for omics	
Microbiome analysis	
Sequencing data analysis	
Statistics for high-dimensional data	
Data sharing and FAIR principles	

I08. Bioinformatics teaching depth. Depth of bioinformatics teaching *[domain BIO]*

Captures: Whether students learn to analyse biological data themselves.

- ☐ (1) None
- ☐ (2) Conceptual only
- ☐ (3) Demonstrations
- ☐ (4) Hands-on analysis of real data
- ☐ (-1) Don't know / Not applicable

I09. Omics publications (3 y). Number of peer-reviewed bioinformatics/omics publications by this department in the past 3 years

Captures: Research output in bioinformatics.

Answer (publications): _____

I10. Data sharing (FAIR). Sharing of omics data in public repositories *[domain BIO]*

Captures: Open-science practice, important for consortium-wide reuse of data.

- ☐ (1) Not shared
- ☐ (2) Shared on request
- ☐ (3) Sometimes deposited in public repositories
- ☐ (4) Routinely deposited with FAIR metadata
- ☐ (-1) Don't know / Not applicable

Part J. Governance, ethics, bias and regulation

What this part captures. Captures the rules around technology use: strategic plans, leadership, oversight, ethics and bias teaching, use of national and international guidance, patient consent, a governance checklist for AI tools (PAHO AI-GUARD) and stakeholder involvement. Strong governance is what makes technology use safe and trustworthy.

Items J01–J07 refer to the named department/course; J08–J12 refer to the policies and procedures that govern it.

J01. Written plan. Does this department/course have a written plan or formal strategic direction for AI and digital technology in teaching? *[domain GOV]*

Captures: Strategic intent written down.

- ☐ (1) None
- ☐ (2) Informal consensus, not documented
- ☐ (3) Draft document under discussion
- ☐ (4) Formal strategic plan, regularly reviewed
- ☐ (-1) Don't know / Not applicable

J02. Leadership commitment. How committed is the leadership of this department (head, division chief) to advancing these initiatives? *[domain GOV]*

Captures: Leadership support, one of the strongest predictors of change.

- ☐ (1) No commitment shown
- ☐ (2) Initial interest, no concrete actions
- ☐ (3) Some commitment, inconsistent or limited in scope
- ☐ (4) Strong commitment with ongoing initiatives and resources
- ☐ (-1) Don't know / Not applicable

J03. Oversight structure. Is there a committee or a named person accountable for AI and digital technology in teaching and clinics? *[domain GOV]*

Captures: Accountability for technology decisions.

- ☐ (1) None
- ☐ (2) Informal responsibility
- ☐ (3) Committee or person named, limited mandate
- ☐ (4) Formal committee with mandate, reporting line and regular meetings
- ☐ (-1) Don't know / Not applicable

J04. AI ethics teaching. Does this course teach AI ethics (bias, privacy, informed consent, accountability, equity)? *[domain GOV]*

Captures: Preparation of students for ethical use.

- ☐ (1) None
- ☐ (2) Brief mention only

- ☐ (3) 1 dedicated session
- ☐ (4) Systematic integration with case-based discussion and assessment
- ☐ (-1) Don't know / Not applicable

J05. Ethics topics. Which ethics topics are covered?

Captures: The ethics content profile.

- ☐ Algorithmic bias and equity
- ☐ Privacy and data protection
- ☐ Informed consent for technology use with patients
- ☐ Professional accountability / liability
- ☐ Transparency and explainability
- ☐ Automation bias / over-reliance
- ☐ Intellectual property and authorship
- ☐ Environmental impact of AI
- ☐ Academic integrity
- ☐ Genetic data and biobank ethics
- ☐ None

J06. Bias teaching. Are students taught to check AI performance across patient subgroups and to recognize biased training data (PAHO Bias-free AI)? *[domain GOV]*

Captures: A specific skill for fair AI.

- ☐ (1) Not taught
- ☐ (2) Mentioned informally
- ☐ (3) 1 dedicated session
- ☐ (4) Hands-on exercises with real or simulated subgroup analyses
- ☐ (-1) Don't know / Not applicable

J07. Ethical guidelines available. Are established ethical guidelines or white papers on AI and digital technology in dentistry available to faculty and students? *[domain GOV]*

Captures: Access to guidance (PAHO/WHO readiness toolkit item).

- ☐ (1) None available
- ☐ (2) Some documents exist but are incomplete
- ☐ (3) Guidelines exist but need dissemination
- ☐ (4) Comprehensive, well-implemented guidelines available
- ☐ (-1) Don't know / Not applicable

J08. Guidance documents used. Which of the following documents are used in teaching or clinical protocols of this department/course?

Captures: Which international and national documents shape practice; the list adapts to the respondent's country.

- ☐ WHO — Ethics and governance of AI for health (2021) and guidance on large multi-modal models (2024/2025)
- ☐ UNESCO — Guidance for generative AI in education and research (2023)
- ☐ IADR / ITU-WHO Focus Group — Core curriculum for oral and dental AI (Schwendicke et al., 2023)
- ☐ AI reporting guidelines (CONSORT-AI, SPIRIT-AI, TRIPOD+AI, STARD-AI, CLAIM, dental AI checklist)
- ☐ PAHO/WHO AI toolkits (Readiness Assessment, AI-GUARD, Bias-free AI, Prompt design) [CA, US, BR]
- ☐ ADA Technical Report 1109, ANSI/ADA Standard 1110-1:2025, ADA White Paper 1106 [US, CA]

- ☐ CODA accreditation standards (AI/technology-related elements) [US]
- ☐ HIPAA privacy and security rules applied to AI tools [US]
- ☐ FDA guidance on AI-enabled device software functions [US]
- ☐ ADEA guidance or competencies relating to AI [US, CA]
- ☐ Canadian Dental Association position statements / guidance on AI [CA]
- ☐ CDAC accreditation standards (technology-related elements) [CA]
- ☐ PIPEDA / provincial health-privacy laws (PHIPA, Quebec Law 25) [CA]
- ☐ Health Canada guidance for machine-learning-enabled medical devices [CA]
- ☐ CFO resolutions / guidance on digital dentistry and AI [BR]
- ☐ LGPD (Lei 13.709/2018) — health data protection [BR]
- ☐ ANVISA RDC 657/2022 — Software as a Medical Device [BR]
- ☐ DCN Odontologia (Resolução CNE/CES 3/2021) [BR]
- ☐ PL 2338/2023 — Brazilian AI regulatory bill [BR]
- ☐ CNS/CONEP research-ethics resolutions (466/2012, 510/2016) [BR]
- ☐ Personal Information Protection Law (2021) / Data Security Law (2021) [CN]
- ☐ NMPA guidelines for registration review of AI medical device software [CN]
- ☐ Interim Measures for the Management of Generative AI Services (2023) [CN]
- ☐ National Health Commission guidance on AI application scenarios in health care [CN]
- ☐ Chinese Stomatological Association expert consensus / guidance on AI [CN]
- ☐ Ministry of Education guidance on AI in higher education [CN]
- ☐ Dental Council of India regulations / National Dental Commission Act 2023 [IN]
- ☐ ICMR Ethical Guidelines for Application of AI in Biomedical Research and Healthcare (2023) [IN]
- ☐ Digital Personal Data Protection Act 2023 and Rules [IN]
- ☐ CDSCO Medical Devices Rules 2017 — software as a medical device [IN]
- ☐ Ayushman Bharat Digital Mission (ABDM) health-data standards [IN]
- ☐ Indian Dental Association guidance on AI [IN]
- ☐ None of these

J09. Regulatory guidance incorporated. Is national / professional regulatory guidance on AI and digital technology in dentistry incorporated into teaching and clinical protocols? *[domain GOV]*

Captures: Translation of regulation into practice.

Country adaptation: see table "Country context", row "Professional and regulatory bodies".

- ☐ (1) Not incorporated
- ☐ (2) Aware but not incorporated
- ☐ (3) Discussed in faculty meetings
- ☐ (4) Incorporated into teaching and clinical protocols
- ☐ (-1) Don't know / Not applicable

J10. Patient consent. Are patients informed / asked for consent when AI or robotic systems are used in their care in the school clinic? *[domain GOV]*

Captures: Respect for patient autonomy.

- ☐ (1) Never
- ☐ (2) Occasionally, at clinician discretion
- ☐ (3) Required by policy but inconsistently applied
- ☐ (4) Required by policy, documented in the record
- ☐ (-1) Don't know / Not applicable

J11. AI tool governance (AI-GUARD). Before and during use of clinical AI tools in this department/course, are the following in place? (adapted from PAHO AI-GUARD, Tier 2)

Captures: A 10-item governance checklist scored Yes = 2, Partial = 1, No = 0 into the AI-GUARD index.

Item	Yes	Partial	No	Don't know
Defined owner / responsible authority for each AI tool	☐	☐	☐	☐
Vendor documentation or model card reviewed (intended use, training data, limitations)	☐	☐	☐	☐
Independent / external validation evidence reviewed	☐	☐	☐	☐
Regulatory clearance verified for the intended use	☐	☐	☐	☐
Performance across patient subgroups evaluated	☐	☐	☐	☐
Human review required before acting on AI output	☐	☐	☐	☐
Clinician override possible and logged	☐	☐	☐	☐
Incident / error reporting process defined	☐	☐	☐	☐
Update / version-change notification procedure defined	☐	☐	☐	☐
Ongoing performance monitoring (drift) planned	☐	☐	☐	☐

Scoring: Yes = 2, Partial = 1, No = 0.

J12. Stakeholder involvement. How involved are stakeholders (faculty, students, clinical staff, IT, patients, external partners) in decisions about technology integration? *[domain GOV]*

Captures: Participation in decisions (PAHO/WHO readiness toolkit item).

- ☐ (1) Not involved
- ☐ (2) Limited, non-systematic involvement
- ☐ (3) Involvement exists but inconsistent
- ☐ (4) Well-established processes for systematic stakeholder input
- ☐ (-1) Don't know / Not applicable

Part K. Implementation, evaluation and sustainability

What this part captures. Measures whether initiatives move from pilots to routine use and whether they last: pilots, scalability, workflow integration, metrics, evaluation, feedback, cost-benefit, sustainability and funding (PAHO/WHO readiness toolkit). It separates one-off enthusiasm from lasting change.

Refers to technology initiatives of the named department/course (AI, informatics, robotics, bioinformatics).

K01. Pilot projects. Name any technology pilot project currently running in this department/course

Captures: Named pilots, their domain and area, to share lessons across schools.

#	Project	Domain	Area
1			
2			
3			

Domain codes: 1 = AI; 2 = Health informatics; 3 = Robotics / digital dentistry; 4 = Bioinformatics

Area codes: 1 = Teaching / learning; 2 = Assessment; 3 = Clinical care; 4 = Research; 5 = Administration

K02. Pilot maturity. Status of technology pilot projects *[domain IMP]*

Captures: Evidence from pilots.

- ☐ (1) No pilot projects

- ☐ (2) Initial pilots with limited outcomes
- ☐ (3) Pilots ongoing, outcomes not yet assessed
- ☐ (4) Pilots with positive, documented outcomes
- ☐ (-1) Don't know / Not applicable

K03. Scalability. Potential for scaling successful initiatives to other courses *[domain IMP]*

Captures: Whether success can spread.

- ☐ (1) No potential
- ☐ (2) Limited potential; substantial effort required
- ☐ (3) Some potential, requires significant resources and coordination
- ☐ (4) High potential with a clear pathway
- ☐ (-1) Don't know / Not applicable

K04. Workflow integration. How well are these technologies integrated into clinical and teaching workflows? *[domain IMP]*

Captures: Routine use versus side projects.

- ☐ (1) Not integrated
- ☐ (2) Used outside routine workflow
- ☐ (3) Partly integrated
- ☐ (4) Fully integrated in routine workflow
- ☐ (-1) Don't know / Not applicable

K05. Performance metrics. Are there established metrics for monitoring the performance or impact of technology teaching initiatives? *[domain IMP]*

Captures: Measurement of results.

- ☐ (1) No metrics
- ☐ (2) Limited metrics
- ☐ (3) Metrics exist but need refinement
- ☐ (4) Comprehensive metrics with regular review
- ☐ (-1) Don't know / Not applicable

K06. Evaluation framework. Is the impact of technology teaching evaluated with a framework (e.g., Kirkpatrick levels: reaction, learning, behaviour, results)? *[domain IMP]*

Captures: Depth of evaluation.

- ☐ (1) Not evaluated
- ☐ (2) Student satisfaction only
- ☐ (3) Learning outcomes measured
- ☐ (4) Learning and clinical/behavioural outcomes measured
- ☐ (-1) Don't know / Not applicable

K07. Feedback mechanisms. Are there mechanisms to collect and act on feedback from students and faculty? *[domain IMP]*

Captures: Listening to users.

- ☐ (1) None
- ☐ (2) Informal feedback only
- ☐ (3) Formal collection, inconsistent follow-up
- ☐ (4) Formal collection with documented actions
- ☐ (-1) Don't know / Not applicable

K08. Continuous improvement. Are there mechanisms for continuous improvement based on evaluation? *[domain IMP]*

Captures: Learning from evaluation.

- ☐ (1) No mechanisms
- ☐ (2) Limited mechanisms
- ☐ (3) Mechanisms exist but need better integration
- ☐ (4) Comprehensive feedback loops
- ☐ (-1) Don't know / Not applicable

K09. Cost-benefit evaluation. Has the cost and benefit of technology initiatives been evaluated (licences, hardware, staff time vs. outcomes)? *[domain IMP]*

Captures: Economic evidence for investment decisions.

- ☐ (1) Never
- ☐ (2) Informal estimate
- ☐ (3) Formal evaluation for some initiatives
- ☐ (4) Formal evaluation for all major initiatives
- ☐ (-1) Don't know / Not applicable

K10. Financial sustainability. Are there plans for long-term financial sustainability (budget lines, grant strategies, partnerships)? *[domain IMP]*

Captures: Whether initiatives will survive after initial funding.

- ☐ (1) No plans
- ☐ (2) Initial discussions only
- ☐ (3) Plans exist but incomplete
- ☐ (4) Comprehensive, well-developed plans
- ☐ (-1) Don't know / Not applicable

K11. Funding sources. Name each funding source that supports technology teaching or research in this department/course

Captures: Who pays, to identify funding opportunities for consortium projects.

#	Funding source	Type
1		
2		
3		

Type codes: 1 = Institutional budget; 2 = National research agency (e.g., NIH, CIHR, CNPq/CAPES/FAPs, NSFC, ICMR/DST); 3 = Industry; 4 = Philanthropy / foundation; 5 = International agency; 6 = Other

K12. Funding adequacy. Funding available for technology teaching or research in this department/course *[domain IMP]*

Captures: Adequacy of money.

- ☐ (1) No funding
- ☐ (2) Limited funding with significant gaps
- ☐ (3) Funding available but inconsistent or insufficient
- ☐ (4) Sufficient, reliable funding
- ☐ (-1) Don't know / Not applicable

K13. Annual budget. Approximate annual budget dedicated to these technologies in this department/course (USD equivalent)

Captures: Order of magnitude of investment, comparable across countries.

- ☐ None
- ☐ Less than 5,000
- ☐ 5,000 – 25,000
- ☐ 25,001 – 100,000
- ☐ More than 100,000
- ☐ Don't know

K14. Grants (3 y). Number of competitive grants related to these technologies obtained by this department in the past 3 years

Captures: Success in attracting competitive funding.

Answer (grants): _____

Part L. Collaboration and the open dental AI model

What this part captures. Maps which departments could contribute data, annotation, validation, teaching or grant work to the consortium's free dental AI model, under which conditions, and what stands in the way. It turns the survey into a practical plan for collaboration.

ICARB-D aims to build a free, open AI model for dentistry through joint work. These items are about your interest and capacity to take part. Answering does not commit your institution.

L01. Interest in ICARB-D projects. Interest of this department in joining ICARB-D collaborative projects

Captures: Who wants to join, to plan the first collaborative projects.

- ☐ Not interested
- ☐ Would like more information
- ☐ Interested in participating
- ☐ Already involved

L02. Data that could be contributed. Which de-identified data could this department potentially contribute to a shared dental AI model (subject to ethics approval)?

Captures: The potential data pool for the open AI model, by modality.

- ☐ Periapical / bitewing radiographs
- ☐ Panoramic radiographs
- ☐ CBCT
- ☐ Intraoral scans (3D)
- ☐ Clinical photographs
- ☐ Structured clinical records
- ☐ Histopathology images
- ☐ Omics / microbiome data
- ☐ None

L03. Images available for sharing. Approximate number of de-identified images or records that could be shared (0 if none)

Captures: Scale of potential contribution.

Answer (images/records): _____

L04. Ability to share data. Ability to share de-identified data with external partners

Captures: How quickly data could actually be shared (not part of the maturity score).

- ☐ (1) Not possible
- ☐ (2) Possible only with a new ethics approval
- ☐ (3) Possible under existing approvals and data-sharing agreements
- ☐ (4) Already sharing data with external partners
- ☐ (-1) Don't know / Not applicable

L05. Preferred ways to collaborate. How would this department prefer to collaborate?

Captures: The collaboration models each partner accepts, e.g., federated learning when data cannot leave the institution.

- ☐ Federated learning (data stays at our institution)
- ☐ Contributing de-identified data to a central repository
- ☐ Annotating / labelling images
- ☐ Testing and validating models on our own data
- ☐ Sharing curricula and teaching materials
- ☐ Joint grant applications and publications

Other, specify: _____

L06. Barriers to sharing. Main barriers to sharing data or collaborating

Captures: Obstacles the consortium must solve (e.g., model agreements, federated tools).

- ☐ Privacy law / legal uncertainty
- ☐ Ethics approval
- ☐ Institutional policy
- ☐ Technical capacity
- ☐ Data quality or lack of annotation
- ☐ Intellectual property concerns
- ☐ Staff time
- ☐ No major barriers

Other, specify: _____

L07. People available. Number of faculty, staff or graduate students who could dedicate time to consortium projects

Captures: Human capacity available for joint work.

Answer (people): _____

L08. Project ideas. Ideas for collaborative projects (optional)

Captures: Suggestions from the field to shape the consortium agenda.

Part M. Self-assessment and open response

What this part captures. Asks the respondent for an overall judgement of maturity in each of the four domains, the main barriers and facilitators, and open comments. Self-ratings are compared with the scores computed from Parts C–K, and open answers explain the numbers.

M01. Overall maturity — AI. Overall maturity of this department/course in artificial intelligence

Captures: Respondent's overall judgement for AI, compared with computed scores.

- ☐ (1) Exploring: no formal action; individual interest only
- ☐ (2) Planning: discussions underway, some resources identified, no systematic implementation
- ☐ (3) Implementing: some formal courses/projects launched, not scaled or sustainable
- ☐ (4) Mature: systematic integration, dedicated resources, continuous evaluation
- ☐ (-1) Don't know / Not applicable

M02. Overall maturity — health informatics. Overall maturity in health informatics

Captures: Overall judgement for health informatics.

- ☐ (1) Exploring: no formal action; individual interest only
- ☐ (2) Planning: discussions underway, some resources identified, no systematic implementation
- ☐ (3) Implementing: some formal courses/projects launched, not scaled or sustainable
- ☐ (4) Mature: systematic integration, dedicated resources, continuous evaluation
- ☐ (-1) Don't know / Not applicable

M03. Overall maturity — robotics. Overall maturity in robotics and digital dentistry

Captures: Overall judgement for robotics and digital dentistry.

- ☐ (1) Exploring: no formal action; individual interest only
- ☐ (2) Planning: discussions underway, some resources identified, no systematic implementation
- ☐ (3) Implementing: some formal courses/projects launched, not scaled or sustainable
- ☐ (4) Mature: systematic integration, dedicated resources, continuous evaluation
- ☐ (-1) Don't know / Not applicable

M04. Overall maturity — bioinformatics. Overall maturity in bioinformatics and omics

Captures: Overall judgement for bioinformatics.

- ☐ (1) Exploring: no formal action; individual interest only
- ☐ (2) Planning: discussions underway, some resources identified, no systematic implementation
- ☐ (3) Implementing: some formal courses/projects launched, not scaled or sustainable
- ☐ (4) Mature: systematic integration, dedicated resources, continuous evaluation
- ☐ (-1) Don't know / Not applicable

M05. Primary barriers. Primary barriers to progress in this department/course (select up to three)

Captures: The three biggest obstacles, ranked across schools and countries.

Select up to 3.

- ☐ Lack of funding
- ☐ Lack of faculty expertise
- ☐ Lack of time in the curriculum
- ☐ Lack of leadership support
- ☐ Lack of equipment / infrastructure
- ☐ Ethical / legal uncertainty
- ☐ Lack of student interest or preparedness
- ☐ Lack of data access
- ☐ Accreditation / curriculum rules leave no room
- ☐ Language barriers (tools/materials not in local language)

Other, specify: _____

M06. Facilitators. Main facilitators of progress (select up to three)

Captures: What helps, to replicate success elsewhere.

Select up to 3.

- ☐ Faculty champions
- ☐ Leadership support
- ☐ Dedicated funding
- ☐ Interdisciplinary partners
- ☐ Industry partnerships
- ☐ Student demand
- ☐ Accreditation / national guidelines
- ☐ National or international networks

Other, specify: _____

M07. Most important need. What is the single most important resource or policy support this department/course needs in the next 2 years?

Captures: The top priority in the respondent's words, analysed by theme.

M08. Innovative practice. Describe any innovative teaching practice with AI, informatics, robotics or bioinformatics in this department/course (optional)

Captures: Good practices to showcase and share.

M09. Vision 2030. How do you expect these technologies to change teaching in this department/course by 2030? (optional)

Captures: Expectations for the future (PAHO "Vision 2030" question).

M10. Confidence in answers. How confident are you in the accuracy of your answers?

Captures: Data-quality flag used in sensitivity analyses.

- ☐ (1) Low
- ☐ (2) Moderate
- ☐ (3) High
- ☐ (4) Very high (checked with records)

M11. Follow-up interview. Would you agree to a short follow-up interview?

Captures: Willingness for qualitative follow-up.

- ☐ Yes
- ☐ No

3. Scoring and consolidation

- **Course record.** Each form is stored as one record keyed by Survey year × School × Department × Course code. Nothing is aggregated before storage.
- **Domain scores.** For each record, the mean of the 1–4 items of each of the ten domains is computed ("Don't know" = missing) and converted to a 0–100 index: (mean − 1) / 3 × 100. The overall index is the mean of the domain indices.
- **AI-GUARD and safeguard indices.** Items J11 and F06 are scored Yes = 2, Partial = 1, No = 0 and expressed as a percentage of the maximum over answered rows.
- **Hours.** Hours per year = hours per term × terms per year (B05), for AI (E02, E03, E09), health informatics (G09), robotics (H05) and bioinformatics (I07).
- **Equipment.** Equipment counts (H01) are summed and divided by enrollment (B01) to give students per unit.
- **Department roll-up.** Scores are averaged across the courses of a department in the same year; the number of contributing courses (n) is reported.
- **School roll-up.** Department scores are averaged with weights equal to the number of courses each department contributed.
- **Country summary.** School scores are averaged per country and year; the number of schools is reported.
- **Annual trends.** The overall index of each department is tabulated by survey year to measure progress between waves.
- **Transparency rule.** For each school, the list of named courses that produced the curriculum score (Part E) is published.

Data file

Answers are stored in a flat file with one row per department/course (access code) and one column per item — 482 answer columns in total. The codebook (docs/codebook.csv) lists every column, its label, type, domain and value codes. Personal contact data (respondent name, email, interviewer) are kept in a separate private file.

Methodological recommendations

- Translation: forward–backward translation into French (Quebec), Portuguese (Brazil) and Chinese, following WHO guidance, with reconciliation by bilingual dental educators. English is used in India and the rest of Canada and the USA.
- Content validity: expert panel (at least two experts per country and per domain) rating item relevance; item-level and scale-level content validity index (I-CVI ≥ 0.78, S-CVI/Ave ≥ 0.90), optionally with a two-round Delphi.
- Pilot: cognitive interviews with 2–3 department heads per country, then a pilot with about 10 departments to check timing and item clarity.
- Reliability: Cronbach's alpha / McDonald's omega per domain; test–retest on a 10% subsample after 2–4 weeks; agreement between two respondents of the same department where possible.
- Sampling frame: official lists of dental schools (ADEA and CODA in the USA, ACFD in Canada, MEC/e-MEC and ABENO in Brazil, MOE and the Chinese Stomatological Association in China, DCI/NDC in India). Stratify by country, public/private status and region; within each school, invite departments teaching imaging, restorative/prosthodontics, orthodontics, periodontics, oral surgery/implantology, oral biology, public health and research methods, plus any department teaching a dedicated technology course.
- Annual design: the same departments are invited every year (panel), new schools join as the consortium grows. Keep item IDs stable between waves; record any change in a versioned codebook.

- Ethics: approval from the coordinating research ethics board and local approval where required (e.g., CONEP in Brazil, institutional ethics committees in India and China). Respondents provide informed consent (item A14). Personal contact data are stored separately from answers.
- Reporting: STROBE for cross-sectional studies and CHERRIES for web-based surveys.
- Analysis: descriptive statistics by country, level and year; Kruskal–Wallis / Mann–Whitney tests for between-country and public/private comparisons; multilevel (mixed-effects) models with courses nested in departments and schools, and survey year as a repeated measure; heat maps of domain indices.

4. Related studies

Most published studies on AI in dental education measure perceptions, knowledge and attitudes of students or educators, often at one institution. Curriculum-document analyses exist for some countries (e.g., Brazilian master's programs), and health informatics, robotics and bioinformatics are rarely surveyed at all. No multinational study has measured digital maturity at the level of named departments and courses, combining physical and human resources, teaching hours by topic, governance and sustainability, and repeating the measurement every year. This survey fills that gap and includes China and India, which are underrepresented relative to the size of their dental workforce.

Study	Setting / design	Key finding
PAHO/WHO & IDB, 2024	Readiness toolkit	Defines the readiness dimensions and the scoring/analysis steps adapted here.
PAHO/WHO & IDB, 2026 (AI-GUARD)	Governance tool	Tiered Yes/Partial/No governance checklist for AI tools; basis of items F06 and J11.
PAHO, 2026 (Bias-free AI)	Guidance	Representative data, subgroup performance reporting and human oversight; basis of items C15 and J06.
PAHO, 2025 (Prompt design)	Guidance	Prompt design as a core digital-literacy competency; basis of items F04–F05.
Schwendicke et al., 2023	Expert consensus (IADR, ITU/WHO FG-AI4H), Delphi	Core curriculum for dental AI in four domains, mostly at "knowledge" level; reference framework for item E20.
Uribe et al., 2024	Worldwide survey of 428 dental educators	31% already used AI chatbots, 64% saw potential; main concern was the lack of guidelines and training.
Kavadella et al., 2026	Multinational survey of dental students	Students want AI education (about 70%) but over half feel unprepared by current curricula.
El-Hakim et al., 2025	Scoping review, 17 studies	AI improves procedural accuracy and feedback but struggles with dexterity and judgment; VR often mislabelled as AI.
Ardabil, Iran, 2025	MAIRS survey, faculty vs students	Faculty readiness higher than students (65.9 vs 52.0 total; ≈3.0 vs 2.4 per item).
Dantas & da Silva, 2026	Curriculum analysis, 92 Brazilian master's programs	AI content in well under 1% of disciplines, concentrated in higher-rated programs.
Wu et al., 2026 (China)	343 students, Wuhan University	89.8% support AI in dental care, 84.2% want AI education; views differ by training stage.
Chongqing, 2026 (China)	Student survey	Use of AI tools and mobile learning apps among dental students.
North India, 2025	Faculty survey, orthodontics and other departments	Faculty see opportunities but report training gaps and ethical concerns.
Sheng et al., 2026 (USA)	243 faculty, residents, students; UTHealth Houston	66% of faculty and 73% of students use LLM tools; faculty report greater need for AI training.
Rifaat et al., 2026	333 students, 55 faculty; Saudi Arabia	Faculty reported far more AI knowledge than students (81.8% vs 27.9%); both want formal training.

Study	Setting / design	Key finding
Busch et al., 2024	4,596 medical, dental, veterinary students; 192 faculties, 48 countries	Positive attitudes and demand for more AI teaching; only 205 dental students.
Teng et al., 2022 (Canada)	Countrywide survey of health-care students	Students want AI in curricula; informs Canadian comparison.
Bisdas et al., 2021	3,133 medical and dental students, 63 countries	72% see AI as a partner rather than a competitor.

Suggested article titles

- Mapping the Digital Maturity of Dental Education: An Annual, Course-Level Survey of Artificial Intelligence, Health Informatics, Robotics and Bioinformatics in Canada, the United States, Brazil, China and India
- Which Course, Which Department? Physical and Human Resources for Artificial Intelligence, Health Informatics, Robotics and Bioinformatics in Dental Schools of the Americas and Asia — Baseline Results of the ICARB-D Annual Survey
- From Awareness to Implementation: Department-Level Maturity of Digital Technologies in Dental Education in Five Countries — A Cross-Sectional Study Using an Adapted PAHO/WHO Readiness Framework

Candidate journals

Journal of Dental Education (ADEA) · European Journal of Dental Education · BMC Medical Education · Journal of Dentistry · International Dental Journal (FDI) · JMIR Medical Education · Frontiers in Dental Medicine · Brazilian Oral Research.

References

1. Pan American Health Organization, Inter-American Development Bank. Artificial Intelligence in Public Health: Readiness Assessment Toolkit — Assessing and Enhancing Preparedness for AI Integration in Public Health. Washington, D.C.: PAHO; 2024.
2. Pan American Health Organization, Inter-American Development Bank. AI-GUARD Tool: Artificial Intelligence Governance & Use Assessment for Responsible Deployment. Washington, D.C.: PAHO; 2026.
3. Pan American Health Organization. Bias-free artificial intelligence in health: Dos and don'ts for developing and implementing algorithms. Washington, D.C.: PAHO; 2026. PAHO/EIH/IS/26-0001.
4. Pan American Health Organization. AI prompt design for public health: Using generative AI responsibly. Washington, D.C.: PAHO; 2025. PAHO/EIH/IS/25-0008.
5. Schwendicke F, Chaurasia A, Wiegand T, et al. Artificial intelligence for oral and dental healthcare: Core education curriculum. J Dent. 2023;128:104363. doi:10.1016/j.jdent.2022.104363
6. Uribe SE, Maldupa I, Kavadella A, et al. Artificial intelligence chatbots and large language models in dental education: Worldwide survey of educators. Eur J Dent Educ. 2024;28(4):865-876. doi:10.1111/eje.13009
7. Kavadella A, Uribe SE, Schwendicke F, et al. Dental students' knowledge, perceptions, and educational needs regarding artificial intelligence: a multinational cross-sectional survey. BMC Med Educ. 2026;26:401. doi:10.1186/s12909-026-08699-6
8. El-Hakim M, Anthonappa R, Fawzy A. Artificial Intelligence in Dental Education: A Scoping Review of Applications, Challenges, and Gaps. Dent J (Basel). 2025;13(9):384. doi:10.3390/dj13090384
9. Ahmadian Yazdi N, et al. Readiness to use artificial intelligence: a comparative study among dental faculty members and students. BMC Med Educ. 2025;25:1006. doi:10.1186/s12909-025-07621-w [verify author list]
10. Dantas JN, da Silva MAD. AI Integration in Brazilian Master's Curricula and Its Implications in Dental Education. Int J Dent. 2026;9974810. Preprint: Research Square rs-3395718. [verify final citation]
11. Wu Y, Bao Y, Jiang Z, et al. Preparing future dentists for artificial intelligence: a cross-sectional study of perceptions and educational needs in China. Front Public Health. 2026;14:1841849. doi:10.3389/fpubh.2026.1841849

12. A survey-based study on dental students' perceptions and attitudes toward AI and mobile medical learning apps in Chongqing, China. BMC Med Educ. 2026. doi:10.1186/s12909-026-10709-6 [verify authors]
13. Faculty perspectives on integrating artificial intelligence into orthodontic and interdisciplinary dental education: opportunities, challenges, and strategies. Cureus. 2025. PMCID: PMC12540188 [verify authors]
14. Sheng C, McFarland C, Angelov N, Eswaran SVK, Halpin R, Chang J. Evaluating artificial intelligence large language models in dental education: a cross-sectional survey on usage, perceptions, and integration at a U.S. dental school. Front Digit Health. 2026;8:1786363. doi:10.3389/fdgth.2026.1786363
15. Rifaat S, Gaffar B, Faridi MA, et al. Dental Students' and Educators' Perceptions Toward the Adoption of Artificial Intelligence in Dental Education: A Survey-Based Study. Int J Dent. 2026;2026:3313083. doi:10.1155/ijod/3313083
16. Busch F, Hoffmann L, Truhn D, et al. Global cross-sectional student survey on AI in medical, dental, and veterinary education and practice at 192 faculties. BMC Med Educ. 2024. PMID: 39342231
17. Teng M, Singla R, Yau O, et al. Health Care Students' Perspectives on Artificial Intelligence: Countrywide Survey in Canada. JMIR Med Educ. 2022;8(1):e33390.
18. Bisdas S, Topriceanu CC, Zakrzewska Z, et al. Artificial Intelligence in Medicine: A Multinational Multi-Center Survey on the Medical and Dental Students' Perception. Front Public Health. 2021;9:795284.
19. Karaca O, Çalışkan SA, Demir K. Medical artificial intelligence readiness scale for medical students (MAIRS-MS) – development, validity and reliability study. BMC Med Educ. 2021;21:112.
20. Thurzo A, Strunga M, Urban R, Surovková J, Afrashtehfar KI. Impact of Artificial Intelligence on Dental Education: A Review and Guide for Curriculum Update. Educ Sci. 2023;13(2):150.
21. American Dental Association. Technical Report No. 1109 (Image analysis systems using AI); ANSI/ADA Standard No. 1110-1:2025 (Validation dataset guidance, Part 1); White Paper No. 1106 (AI in dentistry). Chicago: ADA.
22. World Health Organization. Ethics and governance of artificial intelligence for health: WHO guidance. Geneva: WHO; 2021.